



PAWSIC

POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

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STANDARDS & ASSESSMENT FRAMEWORK

2026 EDITION

Mobile Wound Group *Checklist*

A standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings.

SECTION II • SETTING-SPECIFIC ADD-ON

Skilled Nursing Facility *SNF*

Complete in addition to Section I: Core Standards.

What's *Inside*

- ✓ Regulatory Alignment SNF-1
- ✓ Survey & QAPI Integration SNF-2
- ✓ MDS & Facility Alignment SNF-3
- ✓ Facility Workflow SNF-4
- ✓ Rules of Engagement SNF-5

Scoring

- 0 Absent
- 1 Emerging
- 2 Established
- 3 Best-in-Class

★ Critical items are flagged throughout to signal elevated regulatory or clinical risk.

HOW TO USE

Complete Section I, then your *setting add-on*

Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV).

SECTION I
Core

SECTION II
SNF

SECTION III
HHA

SECTION IV
ALF/IL

Purpose

The PAWSIC Mobile Wound Group (MWG) Checklist 2026 is a standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings. It is intended to support:

- Wound Providers (WPs) and Wound Provider Groups (WPGs): A tool to support program development or process improvement.
- Healthcare Leaders: A tool for any post-acute site of service to aid in the selection of a WP or WPG.
- Patients/Responsible Parties: A reference guide when selecting a WP or WPG.

Intent

This checklist is solely a guide and covers a variety of competencies for both employed WPs and contracted WPGs. It is intended to be adaptable to the needs specific to a site of service, regulations, and applicable practice acts. Regulations, reimbursement structures, and policies that direct practice and care differ from state to state, payer to payer, and by site of service.

This checklist is not intended to replace your facility/organization's policies and procedures nor any local, state, or federal requirements. The utilization of this form is at your discretion. PAWSIC releases this form to your facility/organization to assist in selecting or guiding a partnership with a WP/WPG.

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Instructions: Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV). Section V is a patient-facing document; clinical reviewers do not need to complete it.

★ **Critical items are flagged throughout the document to signal elevated regulatory or clinical risk requiring priority follow-up**

Assessment Intake

Complete this block before starting each evaluation. All fields are required for traceability.

FIELD	ENTRY	ADDITIONAL INFO
Facility / Organization Name (include prior company names, acquisitions, rebranding etc)		
WP / WPG Being Evaluated		
Reason for Review: (New group, annual review, post-incident, due diligence, etc.)		
WPG/WP patient volume/footprint (number of facilities/states)		
Reviewer Name, Title/Role		
Reviewer Disclosures (MWG competitor, vendor, former employee)		
Reviewer of Evidence Name, Title/Role		
Date of Assessment		
Care Setting (select one)	☐ SNF (Skilled Nursing Facility) ☐ HHA (Home Health Agency) ☐ ALF (Assisted Living) ☐ IL (Independent Living)	

SECTION II

Setting-Specific Add-On: SNF

Skilled Nursing Facility | Complete in addition to all Core sections (Section I) | Skip if care setting is HHA or ALF/IL

SCORING

0 Absent

1 Emerging

2 Established

3 Best-in-Class

★ Critical Items, these need to be summarized separately

Comments/Findings/Actions: Reviewer to complete

Evidence Reviewed: in some cases this may be N/A; in others, the evidence used to support findings should be validated through observation of a legal document, website review, etc.

SNF-1 Regulatory Alignment

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Knowledge and application of F684 (Quality of Care: Arterial, Diabetic Foot Ulcer, Venous Leg Ulcer)			

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Knowledge and application of F686 (Pressure Injury Prevention and Treatment)			
Knowledge and application of F880 (Infection Control)			
Avoidable/Unavoidable framework: how WPs work with team to determine, document, and communicate: support of F686 Critical Element Pathway vs. Skin Failure diagnosis			
Documentation supporting survey readiness			
Familiarity with Critical Element Pathways (CEP)			
Training on Skin Failure / Kennedy Terminal Ulcer / Terminal Lesions			
WP training specific to compliance with Federal Regulations in the SNF setting			

SNF-2 Survey & QAPI Integration

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
WP participation in QAPI meetings: frequency, format, data provided			
F689 (Free of Accidents) given fall/skin-tear interactions, and F726 (Sufficient/Competent Nursing Staff) when assessing how WP integrates with limited nursing			
Role of WP during survey: availability, documentation support, active participation			
Process for addressing survey tags related to wound care provider non-compliance			
Data provided to the facility for QI purposes and compliance reporting			

SNF-3 MDS & Facility Alignment

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Collaboration with MDS coordinator/team			
Wound Dx accuracy, PI Staging accuracy, and alignment with MDS coding practices			

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Documentation to support resident/patient wound-affected mobility consistent with MDS Section GG			
Documentation consistency across disciplines to support accurate MDS coding			
WP training on PDPM implications of wound type and associated factors, and documentation			

SNF-4 Facility Workflow

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Frequency of wound rounds in the facility: rules of engagement, who attends			
Integration with Medical Director, DON, and wound nurse			
Support during outbreaks or high-risk infection periods			
How WPs support wound care in the geriatric patient with multiple comorbidities and frailty			
Management of palliative and hospice patients within the SNF setting			
Survey support: how does the WP support the survey process?			

SNF-5 Rules of Engagement

This sub-section establishes the operational and relational framework governing the working relationship between the mobile wound group and the skilled nursing facility. Clear rules of engagement support compliance, communication, and quality of care.

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Patient identification process: criteria used to identify patients/wound types; who initiates the referral (attending physician, DON, wound nurse); standardized referral form or intake process			
PCP order management: responsibility for obtaining/maintaining PCP orders; process for initial orders, renewals, consents for procedures, and communicating plan-of-care changes to attending physician			

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Visit scheduling and consistency: routine schedule (consistent day and time); how schedule is communicated to staff; how nursing, CNAs, and therapy coordinate care accordingly			
Absence and late-arrival protocols: protocol when provider is running late or cancels; who is notified and within what timeframe; interim wound care measures and escalation pathways during absence			
Facility liaison and rounding support: designated facility liaison to accompany WP during rounds; role in supporting infection control, HIPAA, and formulary compliance; consistent vs. rotating role			
Issue identification, reporting, and resolution: formal protocol when either party identifies a concern; how concerns are documented, escalated, and resolved; expected response timeframe and corrective action tracking			
Telehealth and between-visit support: availability for new admissions, acute wound changes, or between-visit concerns; protocol for initiating consult; hours of availability; documentation in medical record			
EMR integration and documentation timeliness: EMR platform and integration with facility system; expected post-visit documentation timeframe; accessibility to facility staff; interim communication processes			
EMR documentation- is there a clear distinction between date of wound onset vs first visit date?			
WP submits an attestation of medical necessity that ties to the SNF's consolidated billing rules to avoid the SNF being caught in Part A/B billing disputes			
Exit meeting and/or report? How does the WP communicate who was seen, wound type, order changes, issues and other with the center leadership?			

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POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

Post-Acute Wound and Skin Integrity Council (PAWSIC) is a non-profit professional group advocating for best practices in wound care and patient-centered care in the post-acute medical sector.

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